Epistaxis

UP-09 | Universal Patient Care

Quick flow

01 Airway + Position [EMT / AEMT / Paramedic]

Sit upright and lean forward when tolerated • suction as needed • never tilt the head backward

02 Major Facial Trauma / Unprotected Airway? [EMT / AEMT / Paramedic]

Do not place intranasal gauze or a clamp • protect airway • control external bleeding • transition to trauma/airway care

03 Clear Clots + Compress [EMT / AEMT / Paramedic]

Alert atraumatic patient gently clears clots once • compress soft lower nose continuously for 15 minutes

04 Maintain Uninterrupted Pressure [EMT / AEMT / Paramedic]

Manual pressure • commercial nasal clamp • or two tongue depressors taped together at one end

05 Persistent Uncontrolled Bleeding? [AEMT / Paramedic]

AEMT/Paramedic: TXA-saturated anterior gauze • bilateral if uncontrolled or source uncertain • pressure 20 minutes

06 Hypotension? [AEMT / Paramedic]

IV/IO access and fluid only for hypotension • reassess after each bolus

07 Monitor After Bleeding Control + Transport [EMT / AEMT / Paramedic]

Secure pressure first • then cardiac monitoring when indicated without interrupting hemorrhage control

Overview

- Control nasal hemorrhage while protecting the airway, identifying traumatic or posterior bleeding, and recognizing significant blood loss or shock.
- Sustained compression of the soft lower nose is the first-line treatment. Topical TXA on gauze is available to AEMTs and Paramedics for persistent atraumatic anterior bleeding; oxymetazoline is not included because it is not currently on the Claiborne formulary.

Provider-Level Actions

Higher provider levels include the actions listed for the preceding levels.

EMT

- Assess airway patency, ability to handle blood and secretions, respiratory status, perfusion, mental status, bleeding severity, estimated blood loss, associated facial trauma, anticoagulant or antiplatelet use, and complete vital signs. Suction blood and secretions as needed.
- Seat the patient upright and leaning forward when tolerated. Do not tilt the head backward. If the alert, cooperative patient has atraumatic bleeding and protects the airway, have the patient gently clear or blow clots from the nose once before compression.
- Compress the soft lower third of the nose continuously for 15 minutes without releasing pressure to check for bleeding. Pressure may be applied manually, with a commercial nasal clamp, or with two tongue depressors taped together at one end. Position the untaped ends externally over the soft nasal alae; never insert the tongue depressors into the nostrils.
- If major facial trauma, suspected basilar skull fracture, altered mental status with an unprotected airway, or inability to manage blood and secretions is present, do not place intranasal gauze or a nasal clamp. Suction, protect the airway, control external bleeding, and transition immediately to the applicable trauma and airway protocols.

- EMT personnel may assist with equipment and continued external pressure but may not administer topical TXA under UP-09.
- Begin prompt transport for airway compromise, hypotension or shock, suspected posterior bleeding, major facial trauma, persistent bleeding, significant estimated blood loss, or uncontrolled bleeding in a patient taking an anticoagulant or antiplatelet medication.

AEMT

- For persistent atraumatic anterior bleeding after 15 minutes of uninterrupted compression, apply topical TXA to gauze. Adult: TXA 1 g/10 mL total. Pediatric: TXA 15 mg/kg, maximum 1 g total. Saturate an appropriately sized gauze pledget and place it gently in the anterior affected nostril without deep or blind insertion.
- If bleeding is uncontrolled when TXA gauze is applied or the bleeding side cannot be identified, place appropriately sized TXA-saturated gauze gently in both anterior nostrils, dividing the approved total dose between the two sides. Leave gauze tails visible externally and maintain external pressure or the nasal clamp for 20 minutes.
- Do not atomize TXA. Do not perform deep, blind, balloon, commercial, or posterior nasal packing. Do not administer IV/IO TXA solely for isolated epistaxis.
- Establish IV or IO access only for hypotension, significant hemorrhage requiring resuscitation, airway management, or another specific clinical indication. Administer crystalloid only for hypotension according to UP-04: adult 500 mL and pediatric 10 mL/kg, followed by reassessment. Use smaller aliquots and frequent reassessment for CHF, renal failure, liver failure, or known volume overload.
- Bleeding control takes priority over routine cardiac-monitor placement. Once uninterrupted pressure or the clamp is secured, apply cardiac monitoring for hypotension, significant blood loss, suspected posterior bleeding, or clinical instability without interrupting hemorrhage control.

Paramedic

- Perform the approved topical TXA treatment and hypotension-directed vascular access and fluid resuscitation described above.
- For persistent blood in the posterior pharynx, bleeding from both nares despite correct anterior compression, continued brisk hemorrhage, or inability to maintain airway protection, suspect posterior bleeding, maintain suction, prepare advanced airway management, and transport promptly with early receiving-facility notification.
- After hemorrhage-control measures are secured, interpret the cardiac rhythm and manage airway compromise, respiratory failure, shock, or dysrhythmia under the applicable protocol. Do not delay transport for repeated packing attempts.

Indications

- Active, recurrent, or recently controlled nasal bleeding requiring EMS evaluation or treatment.

Contraindications

- Do not place intranasal gauze or a nasal clamp in major facial trauma, suspected basilar skull fracture, an unprotected airway, or inability to manage blood and secretions.
- Do not administer topical TXA to a patient with a known hypersensitivity to tranexamic acid.
- Do not perform blind, deep, balloon, commercial, or posterior nasal packing under UP-09.

Assessment | Focused bleeding and airway assessment

- Determine onset, duration, recurrence, suspected side, trauma, prior treatment, prior nasal surgery or recurrent unilateral bleeding, estimated blood loss, anticoagulant or antiplatelet use, bleeding disorder, intranasal drug use, and symptoms of anemia or shock.
- Assess the ability to handle blood and secretions, airway patency, respiratory status, mental status, skin signs, perfusion, complete vital signs, and the posterior pharynx for ongoing blood.

- After clots are cleared once in an alert atraumatic patient, inspect only the visible anterior nasal opening without probing. Persistent blood in the posterior pharynx or continued bilateral bleeding despite correct compression suggests a posterior source.

Treatment steps

- Position upright and leaning forward when tolerated, suction as needed, and protect the airway. Do not tilt the head backward.
- For an alert, cooperative, atraumatic patient who protects the airway, gently clear clots once, then compress the soft lower nose continuously for 15 minutes without releasing to check.
- Maintain pressure manually, with a commercial nasal clamp, or with two tongue depressors taped together at one end and positioned externally over the soft nasal alae.
- For persistent atraumatic anterior bleeding, an AEMT or Paramedic may apply topical TXA to anterior gauze. Use adult TXA 1 g/10 mL total or pediatric TXA 15 mg/kg to a maximum of 1 g total. If bleeding is uncontrolled or the source is uncertain, divide the total dose between gauze pledgets placed gently in both anterior nostrils. Leave external tails and maintain pressure for 20 minutes.
- Do not atomize TXA or perform blind, deep, balloon, commercial, or posterior packing. Establish IV/IO access and give crystalloid only for hypotension.
- Secure bleeding control before routine cardiac-monitor placement. Once pressure is maintained, monitor patients with hypotension, significant blood loss, suspected posterior bleeding, or instability and transport promptly.

Medications

Tranexamic Acid - topical epistaxis

AEMT/Paramedic: adult TXA 1 g/10 mL total; pediatric TXA 15 mg/kg, maximum 1 g total. Apply to appropriately sized anterior gauze and maintain external pressure for 20 minutes.

- Use only for persistent atraumatic anterior bleeding after 15 minutes of uninterrupted compression.
- If bleeding is uncontrolled when TXA is applied or the side is uncertain, divide the approved total dose between gauze pledgets placed gently in both anterior nostrils.
- Leave gauze tails visible externally. Do not atomize TXA and do not perform deep or blind packing.
- EMT administration is not authorized under UP-09. Do not administer IV/IO TXA solely for isolated epistaxis.

Crystalloid - hypotension only

AEMT/Paramedic: adult 500 mL; pediatric 10 mL/kg. Reassess after each bolus and use smaller aliquots for CHF, renal failure, liver failure, or known volume overload.

- Do not establish vascular access or administer fluid routinely for controlled epistaxis in a hemodynamically stable patient.
- Transition to UP-04 and the applicable hemorrhagic-shock or trauma protocol when significant blood loss or shock is present.

Warnings / precautions

- Airway protection and suction take priority when the patient cannot manage blood or secretions. Do not place gauze or a clamp in a patient with an unprotected airway.
- Never tilt the head backward. Blood entering the posterior pharynx may cause aspiration, vomiting, or failure to recognize ongoing hemorrhage.
- A tongue-depressor clamp is applied only to the exterior soft nasal alae. Never insert a tongue depressor into a nostril or allow rigid pressure over the nasal bridge.
- Do not perform blind, deep, balloon, commercial, or posterior packing. Do not place intranasal gauze or a clamp with major facial trauma or suspected basilar skull fracture.
- Do not delay airway care, hemorrhage control, or transport to obtain cardiac monitoring. Apply monitoring after uninterrupted pressure is secured.

Clinical pearls

- Most epistaxis is anterior. Persistent blood in the posterior pharynx, continued bilateral bleeding, or brisk bleeding despite correct anterior compression should raise concern for a posterior source.
- The effectiveness of topical TXA depends on technique. Use TXA-saturated gauze rather than atomized administration and maintain sustained external pressure.
- Do not interrupt pressure repeatedly to inspect the nose. Use the scene timer to document the full 15-minute compression period and the 20-minute TXA-gauze period.
- Document anticoagulant and antiplatelet medications, bleeding duration, estimated blood loss, compression time, clamp method, TXA total dose and laterality, response, vital-sign trends, and destination notification.

Special populations | Pediatrics - younger than 16 years

- Use TXA 15 mg/kg topically to a maximum total dose of 1 g. Select an appropriately sized gauze pledget, leave an external tail, and divide the total dose if both anterior nostrils are treated.
- Maintain continuous airway observation. Do not place intranasal gauze or a clamp when the child cannot cooperate, cannot protect the airway, or cannot manage blood and secretions.

Special populations | Anticoagulant or antiplatelet use

- Use the same first-line compression and topical treatment. Do not delay immediate bleeding control while seeking medication-reversal information.
- Persistent or recurrent bleeding, hemodynamic change, significant estimated blood loss, or suspected posterior bleeding requires prompt transport and early notification.

Source and references

American Academy of Otolaryngology-Head and Neck Surgery Foundation, Clinical Practice Guideline: Nosebleed (Epistaxis), 2020 | Fatahi M, et al. A meta-analysis on the efficacy of topical tranexamic acid for epistaxis: Does the method of administration affect the success rate? Australasian Emergency Care, 2026 | Tennessee EMS BLS/ALS State Protocol Guidelines 2024-2025 - reviewed for state comparison; no dedicated epistaxis protocol identified | Claiborne Covenant EMS Formulary - current medical-director source workbook | Claiborne County EMS UP-09 source PDF - Epistaxis, revised September 1, 2025 | Claiborne County EMS UP-04 Shock / Hypotension and applicable trauma and airway protocols

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